

Supporting Evidence

Attachment Rescue · Track 1 (Ideas) · Ochiai, Ojiro · all figures reproducible from public data with the linked code

1. CENSUS OF THE FULL POPULATION (FILES LISTED BY NAME, NOTHING DOWNLOADED)

NCI-funded open-access papers, 2016–2022, with supplementary files and no data-repository accession	Count
Papers in population (Europe PMC, funder "NCI NIH HHS"; no text-mined accession for GEO, SRA, dbGaP, EGA, ArrayExpress, PRIDE, BioProject, MetaboLights, PDB, GenBank)	26,228
Papers with at least one spreadsheet / CSV / tab-text attachment	4,759 (18.9%)
Spreadsheet / CSV / tab-text files (Excel 14,448; 15.9 GB). Nat Commun 3,201 · eLife 1,440 · PLoS One 1,052 · Sci Rep 682	15,458
Those papers licensed CC BY or CC0 (files may be re-deposited unchanged)	3,824 (80%)
Grants behind the spreadsheet papers: 4,242 NCI awards; FY2016–2022 parent awards (NIH RePORTER)	\$11.2B
Same census, 300-paper samples from other institutes: NHLBI 14.9% · NIDDK 17.6% · NIMH 18.7% · NIA 20.5% · NINDS 23.8% (NCI by the same yardstick: 17.9%); those five populations total 63,043 papers; all 24 institutes: 124,758 papers. 2024–2025 (post-policy), 600-paper sample: 23.2% still carry a spreadsheet with no accession.	NIH-wide

2. FOUR RANDOM SAMPLES OF 300 PAPERS, EVERY ATTACHMENT OPENED (124 WITH A GEO ACCESSION EXCLUDED)

Cancer type	Analysed	Any dataset*	Native spreadsheet	CC BY / CC0	Dataset files
Glioblastoma	271	53 (19.6%)	41 (15.1%)	41	106
Breast cancer	271	51 (18.8%)	41 (15.1%)	46	132
Colorectal cancer	273	53 (19.4%)	41 (15.0%)	47	112
Leukemia	261	45 (17.2%)	33 (12.6%)	33	112
Pooled	1,076	202 (18.8%)	156 (14.5%, CI 12–17%)	167 (83%)	462

*Dataset = largest table ≥ 50 rows $\times$ 3 cols, or $\geq 1,000$ rows; PDF/Word tables only as numeric grids; review files, reporting summaries and figures excluded. Largest table 152,683 rows. Classifier decisions re-examined on 90 files with the AI assistant, not blind, verdicts published: 60/60 "dataset" calls real; every miss under 50 rows. About half primary measurements, half derived results. Filter check on 500 random papers: 15% reference a repository somewhere; about a fifth of files (BMC, PLOS) already have a publisher Figshare DOI. **Full corpus opened: 25,411 attachments → 9,543 dataset files in 3,736 papers; 7,335 with no identifier anywhere.**

3. TWO EXAMPLES A REVIEWER CAN OPEN

- **121-patient glioblastoma cohort table** — *Acta Neuropathol Commun* 2018, PMC6193287, CC BY. Publisher Figshare DOI 10.6084/m9.figshare.7222268 names no NCI grant; absent from NCI's index.
- **118,273-row drug screen** — *Nat Commun* 2019, PMC6599391, CC BY; no identifier anywhere.

4. RESCUED TABLES PUT BACK TO WORK

- Two rescued TCGA-derived glioma tables joined by patient ID: 244 shared patients agree on survival, IDH and age in >96%; pooled 1,037 reproduce the known IDH survival split (validation only) (demo/).
- ~4,400 patient records in verified patient-level tables, ~500 from institutional cohorts not in TCGA, GEO or dbGaP. **All 9,543 datasets written to a source file in the Index of NCI Studies' exact 27-column schema** (deliverable/); 8 packaged with DataCite metadata.

5. CODE, DATA, SITE (OPEN, MIT)

- Checked: BioStudies, publisher Figshare deposits (BMC, PLOS → NLM Dataset Catalog), NLM FAIR-SMART. None decides dataset-vs-not by content, none is funder-scoped, none reaches NCI's index. Mechanisms (26,228 papers): R01 43% · P30 47% · cooperative 19% · training 18% · SPOR 9%.
- Repository: github.com/OJ-IRO/attachment-rescue — code, raw census, results, verdicts, packages, INS source file.
- **Working tool:** attachment-rescue.vercel.app/app — search all 15,458 buried spreadsheets, filter by year/journal/license, look up any PMC ID, open the file. Evidence page: attachment-rescue.vercel.app

6. PRIOR WORK CITED

- FAIR-SMART (PMID 41066526) · Gobeill 2025 (PMC12371329) · Federer 2018/2022 · Colavizza 2020 · Ghosh 2025 (PMC12782234) · Boja 2026 (PMC13479720) · Boyd 2026 (djag199) · CBIIT INS-Data.